

External systems, APIs and costs — what we need to collect the data

Complete source register for the Medical Access Intelligence Engine, with access method, authentication, cost and licence status

Prepared by Joseph, MN&J Labs · 7 August 2026 · Every entry below was checked against the live source or its official documentation on 6–7 August 2026. Costs that are not published are marked UNKNOWN rather than estimated. Priority tiers follow spec v2 §8.1 (P0 = authoritative for status, P3 = discovery only).

An MVP can be built entirely on free sources. Every paid item in this register is either optional or has a free substitute: OncoKB → CIViC, NCCN → ESMO, commercial market-access databases → the national official sources, which is the product's whole premise.

The two real spend lines are **the medical reviewer** and, much smaller, **LLM tokens**. Everything below is plumbing.

1 · Availability and coverage — Q5, Q6 (the proprietary core)

These are the functions no competitor performs for patients. Everything else in this document is supporting material.

Source	What it gives (Q#)	Access	Auth	Cost	Limits / frequency	Notes and risks
PBS (Australia)	Subsidised status, prescribing & dispensing conditions, price, authority path — Q5, Q6, Q3	REST API + CSV bundle	Subscription key (Azure APIM). Public tier keyless	Free	Monthly, 1st. Public tier 1 req / 20 s shared globally	P0. Verified end to end: 1,061 oncology items, 100% field completeness, 14 change events July→August. robots.txt disallows /api/ and *.zip — automation must use the keyed API, not ZIP scraping.
MOH drug registry (Israel)	Registration no. & date, cancellation + date, indications, health flag — Q4, Q6	Undocumented JSON API (POST SearchByName)	None	Free	Unknown — no published contract	P0 but unofficial. Verified by live query: 27 fields. health varies per product and is very likely basket inclusion, but that is inferred, not documented. No SLA; can change or be blocked without notice. Terms of automated use unverified.
NICE syndication (UK)	Guidance, quality standards — Q6 context	REST API (JSON/XML)	Licence agreement + API key	UNKNOWN — not published	n/a	P0 for UK. Cost obtainable only by application. Not needed unless the UK is a target jurisdiction.
NHS dm+d via TRUD (UK)	Medicines dictionary — normalisation	XML download	Free registration + licence	Free	Regular releases	P0 for UK normalisation.
BDPM (France)	Marketed medicines, reimbursement rate — Q5, Q6	Open data (CSV)	None	Free	Monthly	P0. Does not cover hospital oncology — see ATIH below.
ATIH liste en sus (France)	Hospital-billed oncology drugs — Q5, Q6	Published lists	None	Free	Periodic	P0. France needs three sources for one answer: BDPM + ATIH + ministry early-access tables.

Source	What it gives (Q#)	Access	Auth	Cost	Limits / frequency	Notes and risks
GlobalData POLI / NAVLIN / RxData	Pricing, reimbursement, HTA decisions across 100+ markets — Q5, Q6	Commercial API / platform	Enterprise contract	UNKNOWN — enterprise pricing, priced for pharma	Vendor-defined	Not recommended. This is the data our product surfaces; licensing it wholesale would invert the economics. Listed for completeness and as a benchmark.

2 · Regulatory status — Q4

Source	What it gives (Q#)	Access	Auth	Cost	Limits / frequency	Notes and risks
EMA (EU)	Centrally authorised medicines, EPARs — Q4	Structured downloads (Excel/JSON/CSV/XML/RSS)	None	Free	Updated overnight	P1. No formal REST API but clean structured exports.
openFDA (US)	Approvals, labels, adverse events — Q4	REST API	Free API key	Free	240 req/min; 1,000/day keyless → 120,000/day with key	P1. Generous limits; key is free and instant.
TGA (Australia)	Australian registrations (ARTG) — Q4	Web / published datasets	None	Free	Periodic	P0 for AU. Pairs with PBS: TGA says approved, PBS says subsidised.
MHRA (UK)	UK authorisations — Q4	Web / datasets	None	Free	Periodic	P0 for UK.

3 · Evidence and knowledge bases — Q1, Q2 (integrate, do not rebuild)

Source	What it gives (Q#)	Access	Auth	Cost	Limits / frequency	Notes and risks
PubMed E-utilities	Abstracts, metadata — Q1	REST API	Optional free key	Free	3 req/s → 10 req/s with key	P2. Abstracts may carry third-party copyright; NCBI disclaimer must be shown. For bulk work they direct you to a local copy rather than the API.
Europe PMC	Abstracts + open-access full text — Q1	REST API	None	Free	Continuous	P2.
CIViC	Variant interpretation, clinical significance — Q2, Q3	API + bulk download	None	Free	Continuous	P2. The free alternative to OncoKB. Community-curated, openly licensed.
OncoKB	Variant→therapy levels tied to FDA status — Q2, Q3, Q4	REST API	Licence + registration	Free academic only. Commercial/clinical = FEE, amount UNKNOWN	n/a	Two blockers for us: we are a commercial use, so a paid licence applies; and OncoKB may not be used to train AI/ML models at all. Prefer CIViC unless a licence is negotiated.

Source	What it gives (Q#)	Access	Auth	Cost	Limits / frequency	Notes and risks
NCCN	Oncology guidelines — Q2	XML/JSON API for licensees	Commercial licence	PAID, UNKNOWN — scales with viewers and number of guidelines	n/a	Optional. ESMO is the open substitute . Removes a negotiation from the critical path.
Crossref + Retraction Watch	Retractions and corrections — data integrity	REST API + full CSV dump	None	Free	Updated every working day	P1. Build this in week one — it is free and almost nobody does it.

4 · Trials and alternative access — Q7 (integrate or partner)

Source	What it gives (Q#)	Access	Auth	Cost	Limits / frequency	Notes and risks
ClinicalTrials.gov v2	Trials, eligibility criteria — Q7, Q3	REST API	None	Free	~50 req/min	P1. No key, no signup. v2 replaced the classic API in March 2024 — anything older is stale.
ANZCTR (Australia/NZ)	Regional trial registry — Q7	Web / exports	None	Free	Continuous	P1 for the Australian start.
WHO ICTRP	Global trial registry aggregation — Q7	Download / search	None	Free	Weekly	P1 for cross-border coverage.
myTomorrows	Expanded access + trials, 133 countries — Q7	Partnership / platform	Commercial	Partnership, not a data feed	n/a	Do not rebuild this. 16,900+ patients matched, \$29M raised. Talk to them before building anything in Q7.

5 · Terminology and normalisation (required for matching)

Source	What it gives (Q#)	Access	Auth	Cost	Limits / frequency	Notes and risks
SNOMED CT	Clinical concepts, disease coding	Files via national release centre	Free Affiliate Licence	Free in Member countries — Australia and Israel are both Members	Scheduled releases	Good news for both our candidate jurisdictions. A fee applies only in non-Member countries.
WHO ATC	Drug classification (L01/L02 = oncology)	Website; optional Excel	None	Free online; ~€200 for the Excel file	Annual updates	PBS already ships ATC codes with each schedule — the paid file is convenience only.
RxNorm	Drug naming normalisation	API + downloads	None	Free	Weekly/monthly	US-centric but useful for mapping.
UMLS / MeSH	Terminology, synonym expansion	Downloads + API	Free licence + account	Free	Annual	Account required; licence is free.

Source	What it gives (Q#)	Access	Auth	Cost	Limits / frequency	Notes and risks
ICD-10 / ICD-11	Disease coding	WHO API / files	Registration	Free	Periodic	ICD-11 has a public API.

6 · Cost summary

Line	Cost	Comment
Medical reviewer	\$15–25k / year	~30 cards/week at ~4 min each ≈ 2 h of specialist time weekly. My arithmetic from published rates, not a quote. This dominates every other line by 50–100× and is the real unit cost of the product.
LLM tokens (extraction, translation)	~\$100s / year	~\$0.004 per abstract on Haiku 4.5 (~1,500 in / 500 out). A few thousand papers a year in one indication. Noise.
Hosting + PostgreSQL	~\$50–150 / month	One small instance plus managed Postgres.
All P0 access/coverage sources	\$0	PBS, Israel MOH, BDPM, ATIH — all free. This is the surprise finding.
All regulatory + trial sources	\$0	EMA, openFDA, TGA, ClinicalTrials.gov, ANZCTR, WHO ICTRP.
Terminology	\$0	SNOMED CT free in AU and IL; ATC, RxNorm, UMLS, ICD all free.
OncoKB commercial licence	UNKNOWN	Avoidable — use CIViC.
NCCN licence	UNKNOWN	Avoidable — use ESMO.
NICE syndication licence	UNKNOWN	Only if the UK becomes a jurisdiction.
Legal: GDPR Art. 9 basis + SaMD boundary	Thousands, one-off	Not optional before a pilot.
Patent FTO review (US 11,887,738)	One-off	The only single item that can close the project.

Total recurring third-party data cost for an Australia-first MVP: zero. Not low — zero. Every source needed for Q4, Q5, Q6 and Q7 in Australia is free and official. The budget question is the reviewer, and after that legal review. Software and data are not where this project's money goes.

7 · Licence constraints that change the design

OncoKB may not be used to train AI/ML models

This is explicit and applies to academic and commercial use alike; benchmarking is allowed only with permission. Since the spec uses LLM extraction, this must be recorded in the source registry as a hard flag, not a footnote. CIViC carries no such restriction, which is a second reason to prefer it.

PBS robots.txt disallows /api/, /downloads/ and *.zip

The evaluation ZIP I downloaded falls under that rule. A one-off manual download for assessment differs from production crawling, but the architectural conclusion is unambiguous: automation goes through the registered API with a subscription key.

The Israeli API has no published contract

It works and returns rich structured data, but it is undocumented, has no SLA, and its terms for automated use are unverified. Treat it as a dependency that may vanish, and ask the Ministry directly before relying on it commercially.

PubMed abstracts may carry third-party copyright

NCBI requires their disclaimer be visible in software using E-utilities, and directs bulk users to a local copy rather than hammering the API. This shapes the ingestion design, not just the legal page.

8 · What must be obtained before / alongside coding

#	Action	Owner	Why it is on the critical path
1	Register for a PBS API subscription key	Michael	Production ingestion must not scrape the ZIP. Free, but it is a registration step with a lead time.
2	Secure a named oncology reviewer and their rate	Naum	The binding constraint of the whole project. Without one, the review gate in §8 cannot function no matter how much code exists, and the dominant cost line stays unknown.
3	Patent FTO review of US 11,887,738	Naum / counsel	The only item in this register that can end the project outright.
4	Confirm what the Israeli health flag means, and the terms of automated use	Michael / Naum	Cross-check 20–30 drugs against the official basket list, and ask the Ministry. Determines whether Israel is a viable second jurisdiction.
5	Free Affiliate Licence for SNOMED CT	Michael	Free but must be requested before distribution.
6	Legal review: GDPR Art. 9 basis and the SaMD boundary	Naum / counsel	Required before a pilot with real patients, not before launch.

Sources

- PBS public API and rate limit: data.pbs.gov.au/api/api-public.html · PBS API base: data-api.health.gov.au/pbs/api/v3/ · robots.txt: pbs.gov.au/robots.txt
- PBS schedule CSV bundle (downloaded and parsed, Jul + Aug 2026): pbs.gov.au/publication/schedule/2026/08/2026-08-01-PBS-API-CSV-files.zip
- Israel MOH drug registry JSON API (verified by live query): israeldrugs.health.gov.il/GovServiceList/IDRServer/SearchByName
- OncoKB licensing and AI/ML restriction: faq.oncokb.org/licensing · oncokb.org/api-access
- NCCN permissions and commercial licence: nccn.org/guidelines/submissions-licensing-and-permissions/permission-to-cite-or-use-nccn-content-faq
- NICE syndication API: nice.org.uk/reusing-our-content/nice-syndication-api
- NHS dm+d via TRUD: nhsbsa.nhs.uk/pharmacies-gp-practices-and-appliance-contractors/nhs-dictionary-medicines-and-devices-dmd/release-dmd-files
- BDPM: base-donnees-publique.medicaments.gouv.fr/telechargement · ATIH liste en sus: atih.sante.fr/nomenclatures-de-recueil-de-l-information/medicaments
- EMA medicine data downloads: ema.europa.eu/node/82422 · openFDA authentication: open.fda.gov/apis/authentication

- [ClinicalTrials.gov API v2: clinicaltrials.gov/data-api/about-api](https://clinicaltrials.gov/data-api/about-api) · [NLM v2 release note: nlm.nih.gov/pubs/techbull/ma24/ma24_clinicaltrials_api.html](https://nlm.nih.gov/pubs/techbull/ma24/ma24_clinicaltrials_api.html)
- [PubMed E-utilities usage guidelines: ncbi.nlm.nih.gov/books/NBK25497/](https://ncbi.nlm.nih.gov/books/NBK25497/)
- [Crossref + Retraction Watch: crossref.org/documentation/retrieve-metadata/retraction-watch/](https://crossref.org/documentation/retrieve-metadata/retraction-watch/)
- [SNOMED CT licensing and Members: snomed.org/get-snomed](https://snomed.org/get-snomed) · snomed.org/members
- [WHO ATC/DDD toolkit: who.int/tools/atc-ddd-toolkit/atc-classification](https://who.int/tools/atc-ddd-toolkit/atc-classification)
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